CLINICAL TRIAL FAILURES · MISSED ENDPOINTS · PSYCHIATRY & MENTAL HEALTH PHASE 2,3 · STARTS 2015–2024

NMDA / glutamate: 7 of 11 completed trials missed their primary endpoint

7 completed trials across 3 sponsors and 3 molecules ran to the end and missed their primary endpoint on the sponsor's own posted result; 4 met it. Only 0 of 42 closed trials were stopped early for efficacy or safety, which is why this class is read through its completed trials rather than a stop rate. Most completed trials post nothing readable, so the count is a floor.

7 of 11
completed trials with a readable primary result missed it · 4 met it

3 / 3
sponsors / molecules behind the misses

0.0%
stopped early: 0 of 42 closed trials (95% CI 0.0%–8.4%)

65.6%
of 64 trials have closed · 22 still open or unresolved

7 of 11 **Completed and missed the primary endpoint.** Of 33 completed trials in this segment, 11 posted a primary result on ClinicalTrials.gov that can be read — the sponsor's own comparison held to the sponsor's own threshold, or its statement that the endpoint was missed: 7 missed and 4 met. Counted separately and never in the rate above; a floor, not a rate. 9 of the 31 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

The completed trials that missed

Trial	Ph	Sponsor	Experimental drugs	Started	What the posted result says
NCT03185819	2	Johnson & Johnson	ESKETAMINE	2017-10	Change From Baseline in Children's Depression Rating Scale-Revised (CDRS-R) Total Score at 24 Hours Post First Dose (Day 2) — Difference of LS Means -2.4 (95% CI -9.08 to 4.19), no p-value posted and no threshold stated: the interval is consistent with no effect under any conventional threshold, so not significant.
NCT02951988	3	Naurex, Inc, an affiliate of Allergan plc	RAPASTINEL	2016-11	Time to First Relapse During the First 52 Weeks of the Double-Blind Treatment Period — p 0.5305, no threshold stated; 0.1 or above, so not significant under any conventional threshold. Method: Log Rank.
NCT02943577	3	Naurex, Inc, an affiliate of Allergan plc	RAPASTINEL	2016-11	Change From Baseline in Montgomery-Asberg Depression Rating Scale (MADRS) Total Score at the End of Study — p 0.2398, no threshold stated; 0.1 or above, so not significant under any conventional threshold. Least Squares Mean Difference -1 (95% CI -2.7 to 0.68). Method: Mixed Model Repeated Measures (MMRM).
NCT02943564	3	Naurex, Inc, an affiliate of Allergan plc	RAPASTINEL	2016-11	Change From Baseline in Montgomery-Asberg Depression Rating Scale (MADRS) Total Score at the End of Study — p 0.8862, no threshold stated; 0.1 or above, so not significant under any conventional threshold. Least Squares Mean Difference 0.1 (95% CI -1.5 to 1.73). Method: Mixed Model Repeated Measures (MMRM).
NCT02932943	3	Naurex, Inc, an affiliate of Allergan plc	RAPASTINEL	2016-10	Change From Baseline in Montgomery-Asberg Depression Rating Scale (MADRS) Total Score at the End of Trial — p 0.6482, no threshold stated; 0.1 or above, so not significant under any conventional threshold. Least Squares Mean Difference 0.3 (95% CI -1.07 to 1.72). Method: Mixed Model Repeated Measures (MMRM).
NCT02484456	2	National Institute of Mental Health (NIMH)	AV-101	2015-10	Model Adjusted Means for Hamilton Depression Rating Score — p 0.68, no threshold stated; 0.2 or above, so not significant under any conventional threshold. Method: Mixed Models Analysis.

The 6 most recently started of 7 completed trials that missed. The diligence report lists every one, with the numbers or the sentence each verdict was read from.

How this compares

NMDA / glutamate
0.0%
0/42 closed

all psychiatry & mental health trials with a resolved drug target

3.8%
15/396 closed

All psychiatry & mental health Phase 2,3

2.9%
29/993 closed

What is in this class: 103 distinct experimental drugs across 64 trials (small molecule 53, protein 11, unknown 3). Most tested: KETAMINE (26), ESKETAMINE (21), RAPASTINEL (11), MEMANTINE (3), FLUOXETINE (2). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 0 stops came from 0 sponsor-asset programmes across 0 sponsors. Screened alongside 12 other segments, the rate is within what screening that many classes could produce (q=1); the molecules and trials that failed here are the record either way. One further closed trial here was terminated with no cause recorded in the registry; if it were biological the rate would be 2.4%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	0	21	0.0%	0.0%–15.5%
2018–2020	0	15	0.0%	0.0%–20.4%
2021–2024	0	6	0.0%	0.0%–39.0%

Where the stops sit

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Sponsors with most stops: .

Method
Denominator: ClinicalTrials.gov interventional Phase 2,3 psychiatry & mental health trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 44% of industry psychiatry & mental health trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits
Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 65.6% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The diligence report names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. Free during the beta: enter your work email at **clinicaltrialfailures.com/asset-check** and it opens straight away.

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.